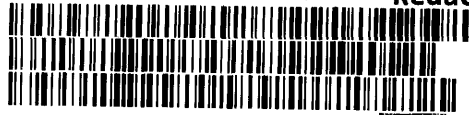


SURGICAL PATHOLOGY REPORT

UUID:A2FF709D-B1C0-4287-8B27-0F96F81BA2CD
TCGA-XF-A8HC-01A-PR

Redacted



Collected

Ordered by

FINAL DIAGNOSIS:

RADICAL CYSTOPROSTATECTOMY WITH EXTENDED PELVIC LYMPHADENECTOMY, CONTINENT URINARY DIVERSION STUDOR POUCH TO URETHRA, G-TUBE PLACEMENT:

RIGHT DISTAL URETER (A):

FROZEN SECTION DIAGNOSIS:

- NO TUMOR IDENTIFIED

FINAL DIAGNOSIS:

- BENIGN URETER

- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

LEFT DISTAL URETER (B):

FROZEN SECTION DIAGNOSIS:

- HYDROURETER, NO TUMOR IDENTIFIED

FINAL DIAGNOSIS:

- HYDROURETER

- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

PARA-AORTIC LYMPH NODES (C):

FROZEN SECTION DIAGNOSIS (CFS 1,2):

- METASTATIC CARCINOMA IDENTIFIED

FINAL DIAGNOSIS:

- METASTATIC CARCINOMA IDENTIFIED IN 4 LYMPH NODES EXAMINED (4/4)

URETHRAL MARGIN (D):

FROZEN SECTION DIAGNOSIS:

- NO TUMOR IDENTIFIED

FINAL DIAGNOSIS:

- BENIGN URETHRA AND PERIURETHRAL TISSUE

- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

BLADDER AND PROSTATE (E):

BLADDER:

- INVASIVE MODERATE TO POORLY DIFFERENTIATED PAPILLARY TRANSITIONAL CELL CARCINOMA, MULTIPLE (0.1 TO 2.5 CM IN GREATEST DIMENSION), GRADE 4/4 INVOLVING DOME, LEFT URETEROVESICAL JUNCTION, TRIGONE AND BLADDER NECK

- TUMOR INFILTRATES THROUGH MUSCULARIS PROPRIA INTO PERIVESICAL FAT

- UROTHELIAL CARCINOMA IN SITU IDENTIFIED

- PERINEURAL INVASION IDENTIFIED

- LYMPHOVASCULAR SPACE INVASION IDENTIFIED

- URETHRAL AND BILATERAL URETERAL RESECTION MARGINS, FREE OF UROTHELIAL DYSPLASIA AND MALIGNANCY

PROSTATE:

- ADENOMATOUS HYPERPLASIA WITH FOCI OF HIGH GRADE PROSTATIC INTRAEPITHELIAL NEOPLASIA (PIN II)

- NO PROSTATIC ADENOCARCINOMA OR INVASIVE PAPILLARY TRANSITIONAL CELL CARCINOMA IDENTIFIED

- BILATERAL SEMINAL VESICLES SHOWING NO TUMOR

OLD SCAR (F):

- SCAR

- NO MALIGNANCY IDENTIFIED

LEFT COMMON ILIAC LYMPH NODES (G):

- METASTATIC CARCINOMA IDENTIFIED IN SEVEN LYMPH NODES EXAMINED (7/7)

ICD O-3

Carcinoma, papillary.
transitional cell 8680/3

Site: 4 Bladder NOS C67.9

Spd 1/23/14

SURGICAL PATHOLOGY REPORT

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PARACAVAL LYMPH NODES (H):

- METASTATIC CARCINOMA IDENTIFIED IN FIVE LYMPH NODES EXAMINED (5/5)

RIGHT COMMON ILIAC LYMPH NODES (I):

- METASTATIC CARCINOMA IDENTIFIED IN EIGHT LYMPH NODES EXAMINED (8/8)

INTRACAVAL LYMPH NODES (J):

- METASTATIC CARCINOMA IDENTIFIED IN TWO OF FOUR LYMPH NODES EXAMINED (2/4)

PARA-AORTIC LYMPH NODES (K):

- METASTATIC CARCINOMA IDENTIFIED IN FOUR LYMPH NODES EXAMINED (4/4)

PARA-AORTIC LYMPH NODES [ABOVE INFERIOR MESENTERIC ARTERY] (L):

- METASTATIC CARCINOMA IDENTIFIED IN 22 OF 23 LYMPH NODES EXAMINED (22/23)

PARA-AORTIC LYMPH NODES [LEVEL OF AORTIC BIFURCATION] (M):

- METASTATIC CARCINOMA IDENTIFIED IN FOUR LYMPH NODES EXAMINED (4/4)

RIGHT EXTERNAL ILIAC LYMPH NODES (N):

- METASTATIC CARCINOMA IDENTIFIED IN ONE OF 12 LYMPH NODES EXAMINED (1/12)

RIGHT LYMPH NODE OF CLOQUET (O):

- NO METASTATIC CARCINOMA IDENTIFIED IN TWO LYMPH NODES EXAMINED (0/2)

RIGHT HYPOGASTRIC / OBTURATOR LYMPH NODES (P):

- METASTATIC CARCINOMA IDENTIFIED IN NINE OF 19 LYMPH NODES EXAMINED (9/19)

LEFT EXTERNAL ILIAC LYMPH NODES (Q):

- METASTATIC CARCINOMA IDENTIFIED IN FOUR OF SIX LYMPH NODES EXAMINED (4/6)

LEFT LYMPH NODE OF CLOQUET (R):

- NO METASTATIC CARCINOMA IDENTIFIED IN SIX LYMPH NODES EXAMINED (0/6)

LEFT HYPOGASTRIC / OBTURATOR LYMPH NODES (S):

- METASTATIC CARCINOMA IDENTIFIED IN 23 OF 30 LYMPH NODES EXAMINED (23/30)

RIGHT PRESACRAL LYMPH NODES (T):

- BENIGN FIBROADIPOSE TISSUE
- NO METASTATIC CARCINOMA IDENTIFIED

PRESACRAL LYMPH NODES (U):

- METASTATIC CARCINOMA IDENTIFIED IN THREE OF FIVE LYMPH NODES EXAMINED (3/5)

LEFT PRESACRAL LYMPH NODES (V):

- METASTATIC CARCINOMA IDENTIFIED IN ONE LYMPH NODE EXAMINED (1/1)

LEFT PERIRECTAL TISSUE (W):

- BENIGN FIBROADIPOSE TISSUE
- NO METASTATIC CARCINOMA OR LYMPH NODES IDENTIFIED

LEFT PROXIMAL URETER (X):

- BENIGN URETER
- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

RIGHT PROXIMAL URETER (Y):

- BENIGN URETER
- NO UROTHELIAL DYSPLASIA OR MALIGNANCY IDENTIFIED

SURGICAL PATHOLOGY REPORT

Collected: [REDACTED]

Ordered by: [REDACTED]

PATHOLOGIC TNM STAGE: T3aN2MX

TOTAL LYMPH NODE COUNT: 97/140

INTRAOPERATIVE DIAGNOSIS:

FROZEN SECTION DIAGNOSIS:

AFS: Right distal ureter:
- no tumor identified

BFS: Left distal ureter:
- hydroureter
- no tumor identified

CFS: Para-aortic lymph nodes:
- metastatic carcinoma identified

DFS: Urethral margin:
- no tumor identified

GROSS DESCRIPTION:

A: The specimen is received fresh from the O.R. and labeled "Right distal ureter". It consists of a segment of cylindrical shaped pink soft tissue measuring 0.5 x 0.3 x 0.1 cm. The specimen is entirely submitted for frozen.

B: The specimen is received fresh from the O.R. and labeled "Left distal ureter". It consists of a segment of cylindrical shaped pink soft tissue measuring 0.8 x 0.5 x 0.2 cm. The specimen is entirely submitted for frozen,

C: The specimen is received fresh from the O.R. and labeled "Para aortic lymph nodes". It consists of fragment of irregularly shaped yellowish fibroadipose tissue measuring 5.0 x 3.0 x 1.0 cm. One possible lymph node is identified measuring 4.5 x 1.0 x 0.5 cm. The possible lymph node is entirely submitted in two blocks for frozen (AFS1 saved for further examination). Remaining tissue entirely submitted in a single cassette.

D: The specimen is received fresh from the O.R. and labeled "Urethral margin". It consists of a fragment of irregularly shaped tan soft tissue measuring 2.3 x 0.3 x 0.3 cm. The specimen is entirely submitted for frozen.

E: The specimen is received fresh from the O.R. and labeled "Bladder & prostate". It consists of a urinary bladder with attached prostate, overall measuring 17 x 13.5 x 4.0 cm. The peritoneum measures 8 x 6.5 x 0.3 cm. The bladder measures 7 x 5 x 2 cm. The peritoneum is grossly unremarkable. The prostate measures 5 x 4.5 x 3.5 cm. The external surface is unremarkable and entirely inked black. The bladder is longitudinally opened anterior to reveal multiple polypoid lesions on the posterior wall. The largest polypoid mass measures 2.3 x 0.6 x 0.5 cm and is located in the dome. Approximately 2.5 cm away from this lesion, another large polypoid mass is also identified in the left posterior wall about 1 cm above the right ureterovesical junction. The second lesion measures 2.5 x 1.7 x 1.5 cm. Multiple small polypoid lesions ranging from 0.1 to 0.3 cm is present as well in the posterior wall, one polypoid lesion is 1.4 cm above the left ureterovesical junction with the remaining polypoid lesions located on the right posterior ureteral wall between the two large lesions. A ulcerated area is also identified in the right posterior wall measuring 1.5 cm in greatest dimension and grossly involves the right ureterovesical junction. This ulceration is 4 cm away from apical margin. Cut sectioning of the tumor shows tan-pinkish and granular appearance. The tumor appears confined to the bladder mucosa and does not infiltrate into the bladder wall. The remaining urinary bladder mucosa shows congestion. The right ureter measures 3 cm in length and 0.6 cm in circumference

SURGICAL PATHOLOGY REPORT

Collected: [REDACTED]

Ordered by: [REDACTED]

and the left ureter measures 4.5 cm in length and 2 cm in circumference. The remaining ureter mucosa is unremarkable. The prostatic urethra measures 3.5 cm in length and 2.5 cm in circumference. The verumontanum measures 1.5 x 0.3 x 0.3 cm. The cut section of the prostate shows multiple nodules involving both left and right lobes, extending from distal to proximal, mainly located in the periurethral region. No definitive carcinoma is identified. No perivesical lymph nodes identified grossly. Representative sections are submitted in 30 cassettes.

F: The specimen is received in formalin and labeled "Old scar". It consists of an ellipse of tan skin measuring 12 x 1 cm and 0.5 cm in thickness. A 11.5 cm linear scar is identified in the center of the specimen. The specimen is serially section and representative section are submitted in a single cassette.

G: The specimen is received in formalin and labeled "Left common iliac lymph nodes". It consists of five possible lymph nodes ranging from 0.5 to 2.8 cm in greatest dimension. The specimen is entirely submitted in two cassettes.

H: The specimen is received in formalin and labeled "Para caval lymph nodes". It consists of a fragment of irregularly shaped tan-yellow fibroadipose tissue measuring 2.5 x 1.8 x 1.0 cm. Two possible lymph nodes are identified ranging from 0.5 to 1.5 cm in greatest dimension. The specimen is entirely submitted in a single cassette.

I: The specimen is received in formalin and labeled "Right common iliac lymph nodes". It consists of two fragments of irregularly shaped tan-yellowish fibroadipose tissue measuring 4.0 x 1.7 x 1.2 cm and 4.0 x 2.5 x 2.0 cm. There are six possible lymph nodes identified ranging from 0.3 to 2.5 cm in greatest dimension. The specimen is entirely submitted in five cassettes.

J: The specimen is received in formalin and labeled "Intra caval lymph nodes". It consists of a fragment of irregularly shaped tan-yellow fibroadipose tissue measuring 3.5 x 2.5 x 0.3 cm. There are four possible lymph nodes identified ranging from 0.3 to 1.0 cm in greatest dimension. The specimen is entirely submitted in a single cassette.

K: The specimen is received in formalin and labeled "Para aortic lymph nodes". It consists of a fragment of irregularly shaped yellowish fibroadipose tissue measuring 2.0 x 1.0 x 0.5 cm. Multiple small nodules are identified ranging from 0.1 to 0.3 cm in greatest dimension. The specimen is entirely submitted in a single cassette.

L: The specimen is received in formalin and labeled "Para aortic (above inferior mesenteric artery)". It consists of a fragment of irregularly shaped tan-yellow fibroadipose tissue measuring 5.0 x 3.0 x 1.0 cm. There are nine possible lymph nodes identified ranging from 0.3 to 3 cm in greatest dimension. The specimen is entirely submitted in five cassettes.

M: The specimen is received in formalin and labeled "Para aortic lymph node (level of aortic bifurcation)". It consists of a fragment of irregularly shaped tan soft tissue measuring 1.5 x 1.0 x 0.3 cm. Multiple nodules are identified ranging from 0.2 to 0.5 cm in greatest dimension. The specimen is entirely submitted in a single cassette.

N: The specimen is received in formalin and labeled "Right external iliac lymph nodes". It consists of multiple fragments of irregularly shaped pink-yellow fibroadipose tissue measuring 3.5 x 2.0 x 1.5 cm. There are three possible lymph nodes identified ranging from 0.3 to 2 cm in greatest dimension. The specimen is entirely submitted in three cassettes.

O: The specimen is received in formalin and labeled "Right lymph node of cloquet".

SURGICAL PATHOLOGY REPORT

Collected: [REDACTED]

Ordered by: [REDACTED]

It consists of a fragment of irregularly shaped tan-yellow fibroadipose tissue measuring 2.0 x 0.7 x 0.7 cm. There is one possible lymph node identified measuring 1 cm in greatest dimension. The specimen is entirely submitted in a single cassette.

P: The specimen is received in formalin and labeled "Right hypogastric/obturator lymph nodes". It consists of multiple fragments of irregularly shaped yellowish fibroadipose tissue measuring 5.5 x 3.0 x 2.0 cm. There are 12 possible lymph nodes identified ranging from 0.3 to 2.5 cm in greatest dimension. The specimen is entirely submitted in five cassettes.

Q: The specimen is received in formalin and labeled "Left external iliac lymph nodes". It consists of multiple fragments of irregularly shaped pink-yellowish fibroadipose tissue measuring 4.0 x 2.5 x 1.5 cm. There are five possible lymph nodes identified ranging from 0.3 to 4 cm in greatest dimension. The specimen is entirely submitted in three cassettes.

R: The specimen is received in formalin and labeled "Left lymph node of cloquet". It consists of a fragment of irregularly shaped yellowish fibroadipose tissue measuring 1.8 x 1.5 x 1.0 cm. There is one possible lymph node identified measuring 1.9 cm in greatest dimension. The specimen is bisected and entirely submitted in a single cassette.

S: The specimen is received in formalin and labeled "Left hypogastric/obturator lymph nodes". It consists of multiple fragments of irregularly shaped pink-yellow fibroadipose tissue measuring 5.5 x 4.0 x 2.5 cm. There are 15 possible lymph nodes identified ranging from 0.2 to 2.0 cm in greatest dimension. The specimen is entirely submitted in three cassettes.

T: The specimen is received in formalin and labeled "Right presciatic lymph nodes". It consists of multiple fragments of irregularly shaped yellowish fibroadipose tissue measuring 2.5 x 2.0 x 0.5 cm. The specimen is entirely submitted in a single cassette.

U: The specimen is received in formalin and labeled "Presacral lymph nodes". It consists of multiple fragments of irregularly shaped tan-yellowish fibroadipose tissue measuring 4.5 x 3.0 x 0.5 cm. There are four possible lymph nodes identified ranging from 0.5 to 1.0 cm in greatest dimension. The specimen is entirely submitted in two cassettes.

V: The specimen is received in formalin and labeled "Left presciatic lymph nodes". It consists of multiple fragments of irregularly shaped yellowish fibroadipose tissue measuring 2.0 x 1.0 x 0.3 cm. There is one possible lymph node identified measuring 0.5 cm in greatest dimension. The specimen is entirely submitted in a single cassette.

W: The specimen is received in formalin and labeled "Left perirectal tissue". It consists of two fragments of irregularly shaped tan-yellowish fibroadipose tissue measuring 1.0 x 0.5 x 0.2 cm and 2.5 x 1.5 x 0.5 cm. No gross abnormalities identified. The specimen is serially sectioned and entirely submitted in a single cassette.

X: The specimen is received in formalin and labeled "Left proximal ureter". It consists of a segment of ureter measuring 0.7 cm in length and 0.4 cm in average diameter. The specimen is serially sectioned and entirely submitted in a single cassette.

Y: The specimen is received in formalin and labeled "Right proximal ureter". It consists of segment ureter measuring 3.8 cm in length and 0.5 cm in diameter. The specimen is serially sectioned with no gross abnormalities identified. The

SURGICAL PATHOLOGY REPORT

Collected

Ordered by

specimen is entirely submitted in a single cassette.

SECTIONS:

AFS: frozen section, right distal ureter all embedded
 BFS: frozen section, left distal ureter all embedded
 CFS1,2: frozen section, para-aortic lymph nodes
 CRT: remaining tissue
 DFS: frozen section, urethral margin all embedded
 E1: right ureter margin
 E2: left ureter margin
 E3: urethra margin
 E4-6: left side multiple polypoid lesions
 E7: right side polypoid lesion
 E8-11: largest lesion at right posterior bladder wall
 E12: dome lesion
 E13: trigone and ulcerated area
 E14: left ureterovesical junction
 E15: right ureterovesical junction
 E16: trigone and ulcer
 E17: trigone, bladder neck and ulcer
 E18: right distal prostate
 E19: left distal prostate
 E20: right anterior mid prostate
 E21: right posterior mid prostate
 E22: left anterior mid prostate
 E23: left posterior mid prostate
 E24: right anterior proximal prostate
 E25: right posterior proximal prostate with seminal vesical junction
 E26: left anterior proximal prostate
 E27: left posterior proximal prostate with seminal vesical junction
 E28-30: perivesical fat with possible lymph nodes
 F: old scar
 G1: two possible left common iliac lymph nodes
 G2: three possible lymph nodes
 H: paracaval lymph nodes all embedded
 I1: four possible right common iliac lymph nodes
 I2: one bisected possible lymph node
 I3: one bisected possible lymph node
 I4: one bisected possible lymph node
 I5: remaining fat
 J: intracaval lymph nodes all embedded
 K: para-aortic lymph nodes all embedded
 L1: two possible para-aortic lymph node (above inferior mesenteric artery)
 L2: four possible lymph nodes
 L3: three possible lymph nodes
 L4: one bisected possible lymph node
 L5: remaining fat
 M: para-aortic lymph nodes (level of aortic bifurcation)
 N1: three possible right external iliac lymph nodes
 N2: remaining tissue
 O: right lymph node of Cloquet all embedded
 P1: six possible hypogastric / obturator lymph nodes
 P2: four possible lymph nodes
 P3: one bisected possible lymph node
 P4: one bisected possible lymph node
 P5: remaining tissue
 Q1: four possible left external iliac lymph nodes

SURGICAL PATHOLOGY REPORT

Temporary

Case

Collected

Ordered by

Q2: one bisected possible lymph node
 Q3: remaining tissue
 R: left lymph node of Cloquet all embedded
 S1: six possible left hypogastric / obturator lymph nodes
 S2: four possible lymph nodes
 S3: four possible lymph nodes and remaining tissue
 T: right presciatic lymph nodes
 U1: four possible presacral lymph nodes
 U2: remaining tissue
 V: left presciatic lymph nodes
 W: left perirectal lymph nodes
 X: left proximal ureter
 Y: right proximal ureter

MICROSCOPIC DESCRIPTION:

A-D: See final microscopic-diagnosis.

E: Sections of the urinary bladder show an invasive papillary transitional cell carcinoma, high grade (4/4), characterized by a papillary architecture of enlarged atypical cells with correspondingly enlarged nuclei and scant cytoplasm. The tumor involves the posterior wall of dome, left ureterovesical junction, trigone and bladder neck. The tumor infiltrates through the muscularis propria into the perivesical fat but does not involve the resection margins. Perineural and lymphovascular invasion identified (E13,17). Also present is urothelial carcinoma in situ involving the mucosa adjacent to the invasive carcinoma.

Sections of the prostate show adenomatous hyperplasia with foci of high grade prostatic intraepithelial neoplasia (PIN II), however no prostatic adenocarcinoma is identified. The prostatic urethra is unremarkable. No invasive papillary transitional cell carcinoma is identified. The bilateral seminal vesicles show no significant abnormalities.

F-Y: See final microscopic-diagnosis.

Signed by:

ELECTRONIC SIGNATURE

POST-OPERATIVE DIAGNOSIS:

Same as pre-op

PRE-OPERATIVE DIAGNOSIS:

Gross Hematuria #10 TCC Bladder

ORDERING PHYSICIAN:

Ordering Physician:

Criteria	Yes	No
Diagnosis Discrepancy		✓
Primary Tumor Site Discrepancy		✓
HPAA Discrepancy		✓
Prior Malignancy History		✓
Local/Synchronous Primary Noted		✓
Case is (circle):	QUALIFIED	DISQUALIFIED
Reviewer Initials	JAC	Date Reviewed: 10/24/2013